

Common Shoulder Injuries and Conditions

Rotator Cuff Injuries

Injury overview	Rotator cuff is a single term that describes the combined function of four muscles (teres major, teres minor, subscapularis, and infraspinatus) that provide stability and support for the glenohumeral joint. Common injuries to this collection of muscles include strains, tendinopathies, and impingement.
Surgical and rehabilitative management	Conservative approaches generally improve function and return athletes to full participation within 3–6 months. If symptoms persist beyond that point, surgical repair should be explored. Rehabilitation includes initial inflammation management, followed by a gradual return to resistance exercises within a pain-free ROM.
Return-to-play exercise and conditioning considerations	Avoid early overhead activities and delay throwing (if applicable) until clearance from the medical team.
Return-to-play criteria	Symptom-free, pain-free ROM, normal strength, and endurance. Sport-specific functional performance battery without complication (including the upper-extremity closed kinetic chain stability test). The Functional Arm Scale for Throwers may be a valuable tool.
Estimated timeline	Variable based on the degree of tissue damage and whether the surgical repair was used. Athletes with minor strains can resume full participation in 1–2 weeks, whereas the timeline for those with more complicated cases could extend to 6 months (or more).

Clavicular Joint Sprains

Injury overview	The clavicle has two articulations within the shoulder complex: sternoclavicular (distal end) and acromioclavicular (proximal end). Increased damage to the supportive ligaments that make up each joint allows for accessory motion leading to subluxations (joint slips in/out of normal position) and complete dislocations (joint slips and remains out). These joint injuries can be problematic for athletes in sports that require high-velocity overhead movements (e.g., volleyball spike, baseball throwing, tennis serve) or endure high-load compressive forces (e.g., football tackle, wrestling), considering the end-range strength and stability required for normal joint function.
Surgical and rehabilitative management	Minor sprains can heal relatively quickly when external joint supports are added (bracing, taping) to progressive exercise programs within pain-free ROMs.
Return-to-play exercise and conditioning considerations	Avoid early overhead activities and delay throwing (if applicable) until clearance from the medical team.

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Return-to-play criteria	Symptom-free, pain-free ROM, normal strength, and endurance. Sport-specific functional performance battery without complication (including the upper-extremity closed kinetic chain stability test). The Functional Arm Scale for Throwers may be a valuable tool.
Estimated timeline	Return to participation is estimated mainly by the degree of damage, whether the surgical repair was required, the contact nature of the sport, and the location of the injury (e.g., athlete's dominant versus nondominant side). Recovery from less severe sprains may require only 1–2 weeks, but that from more severe surgically repaired injuries can exceed 10 weeks.

Biceps Tendinopathy

Injury overview	In athletics, tendinopathies to the biceps tendon primarily involve the long head of the biceps, which has an intra-articular point of origin. This attachment area is prone to compression (by the acromion) during overhead activities, creating an inflammatory response. During overhead throwing motions, the highest activity of the biceps muscle is during the follow-through stage, when the bicep decelerates dynamic elbow extension.
Surgical and rehabilitative management	Return-to-play outcomes are marginal for surgical repair of the long head of the biceps tendon, particularly for those athletes in roles that require hard throwing.
Return-to-play exercise and conditioning considerations	Emphasis on rotational movements (combined flexion/abduction/external rotation followed by extension/adduction/internal rotation, push-up with rotation) over shoulder abduction exercises (lateral raises, overhead press). Continue to progress isolated rotator cuff performance and integrated multisensory exercises for core stability, endurance, and balance.
Return-to-play criteria	Sport-specific functional performance battery without complication (including the upper-extremity closed kinetic chain stability test).
Estimated timeline	2–6 weeks depending on the degree of damage and the overall requirement to participate in hard-throwing-related activities (e.g., baseball, softball, volleyball, tennis).

Superior Labrum Anterior–Posterior (SLAP) Lesions

Injury overview	The labrum is a cartilage tissue that attaches to the glenoid of the humerus. It extends the surface area of the glenoid's small bony surface and passively increases the dynamic joint stability of the glenohumeral joint. A common tear in this tissue is a SLAP lesion. These lesions originate at the superior glenoid (in addition to the long
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	head of the biceps tendon) and run from an anterior-to-posterior direction. Such injuries destabilize the glenohumeral joint and produce pain during high-velocity throwing, requiring rotation ROMs near extreme limits.
Surgical and rehabilitative management	Surgical repair is recommended to restore structural joint stability. Rehabilitation begins following a 3- to 4-week immobilization phase and progresses from that point to improve flexibility, mobility, and stability in a gradual way.
Return-to-play exercise and conditioning considerations	Avoid early overhead activities and delay throwing (if applicable) until clearance from the medical team.
Return-to-play criteria	Normal, pain-free ROM, strength, and endurance. Sport-specific functional performance battery without complication (including the upper-extremity closed kinetic chain stability test).
Estimated timeline	4–6 months, depending on the requirement for the injured area to participate in high-velocity or high-compression activities.